Anxiety, OCD & Stress-Related Disorders

Psychiatry, Clinical Lecture | Dr. Hussain M. Khrad (Consultant Psychiatrist, KFSHRC-Jeddah) | Source: `Anxiety_disorders.pdf` (71 slides)

> **Legend:** ⭐ = high-yield · *[added]* = not on slides. Images marked "Slide N" come from the lecture. All MCQ answers and the case model answer come from the lecture's own answer slides.
>
> 🔗 **Bridge from earlier lectures:**
>
> - **History Taking:** the bilingual screening scripts (GAD, panic, agoraphobia, social anxiety, phobia, OCD) and the rule **"the feared CONSEQUENCE, not the situation, separates the disorders"** are developed fully here. Also: **caffeine** in the substance history.
> - **MSE / Personality Disorders:**
> - **obsession** (ego-dystonic, resisted, insight kept) vs **delusion**
> - **OCD vs OCPD** (ego-dystonic vs ego-syntonic)
> - **avoidant PD ≈ social anxiety disorder**
> - **Mood Disorders: adjustment disorder** (within 3 months, resolves within 6 months); GAD symptoms must occur **outside** MDD.
> - **Endocrine/Cardiology: hyperthyroidism, phaeochromocytoma, hypoglycaemia, arrhythmia, MI, PE** mimic anxiety. Panic patients present to **cardiology** first (the lecture's case).

---

1. Title / Objectives

Domain	Objective	Likely tested as
Knowledge	Define anxiety; when it becomes a disorder	Fear (imminent) vs anxiety (future); Yerkes–Dodson
Knowledge	⭐ The **three DSM-5 chapters** (anxiety · OC-related · trauma/stressor)	The split itself is examinable
Knowledge	⭐ Criteria and **duration thresholds**	GAD 6 months; ASD 3 days–1 month; PTSD > 1 month; OCD > 1 h/day
Knowledge	Obsession vs compulsion; course; first-line management	Checking = compulsion; ERP
Skill	Screen (Arabic/English); ⭐ **ask WHY a situation is feared**	Agoraphobia vs specific phobia
Skill	⭐ **ASD vs PTSD by the clock**	10 days post-crash = ASD
Skill	Exclude substance, medication, medical causes	Caffeine, thyroid, cardiac
Skill	Explain exposure therapy / ERP	Fear ladder; OCD cycle
Values	Anxiety is a disabling illness; shame hides taboo obsessions and hair-pulling; ⭐ **ask about suicide in OCD and BDD**; avoid long-term benzodiazepines; **aim for function, not zero anxiety**	—

⭐ **DSM-5 split the old "anxiety disorders" chapter into three:** (1) **anxiety disorders**, (2) **obsessive-compulsive and related**, (3) **trauma- and stressor-related**.

---

2. Main Content (Lecture Order)

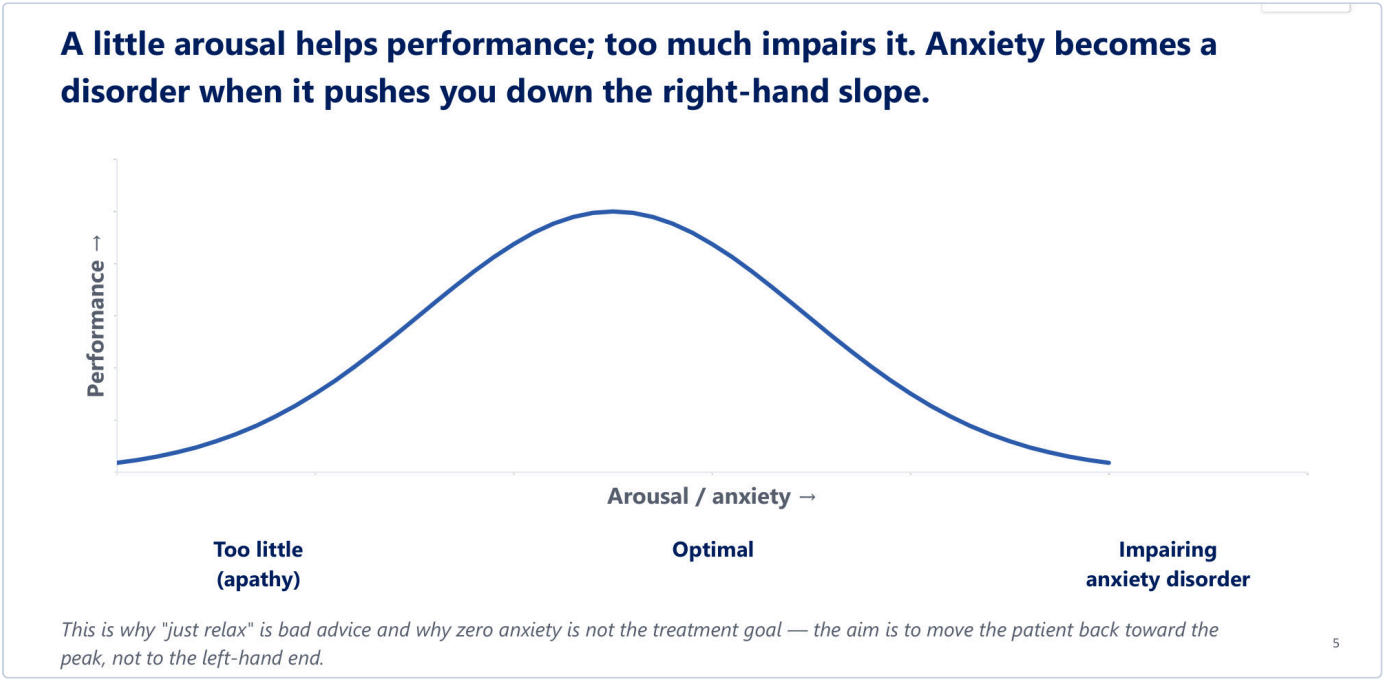
2.1 Foundations (Slides 4–7)

Fear	Anxiety
Response to a real/perceived **IMMINENT** threat	Anticipation of a **FUTURE** threat
Autonomic arousal, **fight-or-flight**	**Muscle tension and vigilance**
Thoughts of immediate danger	Disorders share excessive fear/worry + **avoidance**; they differ in **WHAT is feared**

⭐ **Two universal rules for every diagnosis here:**

1. not due to a **substance, medication or medical condition**
2. causes **clinically significant distress or impairment**

Yerkes–Dodson curve (slide 5): a little arousal improves performance; too much impairs it (the right-hand slope = anxiety disorder). ⭐ **Zero anxiety is not the goal**: move the patient back toward the peak. "Just relax" is bad advice.



Slide 5: Yerkes–Dodson curve

Symptoms by system (slide 6):

System	Manifestations
Constitutional	Fatigue, **diaphoresis**, shivering
Cardiac	**Chest pain, palpitations, tachycardia, hypertension**
Pulmonary	**Shortness of breath, hyperventilation**
Neuro/MSK	Vertigo, light-headedness, **paraesthesias**, tremor, insomnia, muscle tension
GI	Abdominal discomfort, anorexia, nausea, vomiting, diarrhoea, constipation

Patients often reach **cardiology, the ED or GI clinics** long before psychiatry.

Organic and drug causes: always exclude first (slide 7) ⭐

Medications & substances	Medical conditions
⭐ **Caffeine**; stimulants (cocaine, amphetamines)	⭐ **Hyperthyroidism, phaeochromocytoma, hypoglycaemia**
Sympathomimetics (decongestants)	Cardiac/pulmonary: **arrhythmia, MI, PE, asthma, COPD**
Bronchodilators (theophylline, salbutamol), thyroxine, steroids	Neurological: **seizure, vestibular disease**
⭐ **WITHDRAWAL:** alcohol, benzodiazepines, opioids, nicotine	—

New-onset anxiety in an older patient with no history → screen thyroid, cardiac and caffeine first.

2.2 The Anxiety Disorders Map and Screening (Slides 8–10)

Childhood-onset	Common adult	The rest
Separation anxiety disorder	**Social anxiety disorder**	**GAD**
Selective mutism	**Panic disorder**	Substance/medication-induced
Specific phobia	**Agoraphobia**	Due to another medical condition

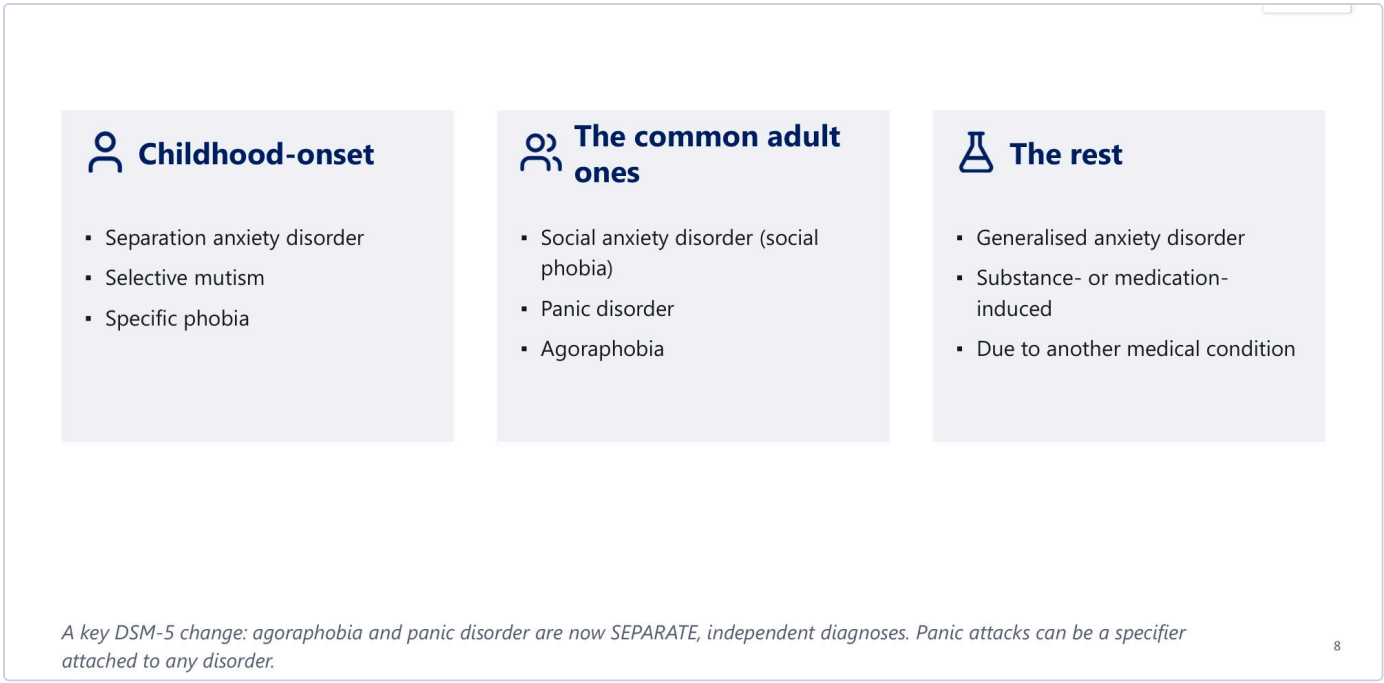
⭐ **DSM-5 change: agoraphobia and panic disorder are now SEPARATE, independent diagnoses.** "Panic attacks" can be a **specifier** on any disorder.

Screening (slide 9):

- **Core screen:** "Do you worry a lot? Sudden rushes of fear? Avoid places? Repetitive thoughts or rituals?"
- **Panic screen:** "A sudden surge of fear peaking within minutes, with palpitations, chest tightness, choking and fear of dying?" (Arabic version on the slide)
- **Then localise:**
  - ⭐ **WHAT is feared and WHY** (the feared consequence)
  - duration and impairment
  - rule out substance/medical causes and depression

Management overview (slide 10):

Psychotherapy	Pharmacotherapy	General
⭐ **CBT first-line** across most disorders	⭐ **SSRIs and SNRIs first-line**	Psychoeducation; **reduce caffeine**
Exposure-based therapy for phobias, panic, agoraphobia	⭐ **Benzodiazepines short-term only, then tapered**	Treat comorbid depression and substance use
ERP for OCD	(Detail in the Psychopharmacology lecture)	**Continue treatment** to prevent relapse



Slide 8: The anxiety disorders

2.3 Generalised Anxiety Disorder (Slides 12–17) ⭐

Case (slides 12–13): a 34-year-old woman with uncontrollable worry about work, children and health, plus restlessness, poor sleep and muscle tension. Minimum duration → ⭐ **D: ≥ 6 months, more days than not.**

Criteria: "WATCHERS" (slide 15):

- excessive anxiety and worry, **more days than not for ≥ 6 months**, about **several** events/activities
- the worry is **difficult to control**
- ⭐ **≥ 3 of 6 symptoms in adults (only 1 in children)**

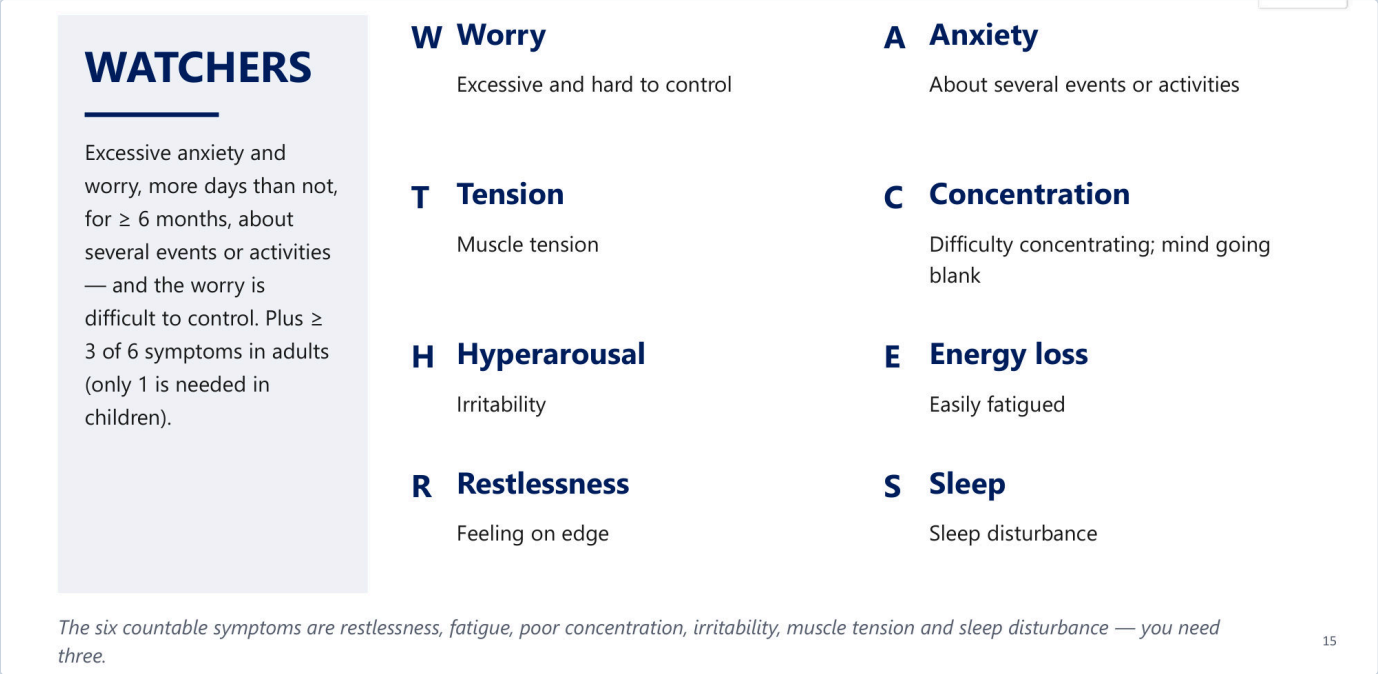
Letter	Feature
W	**Worry**: excessive, hard to control
A	**Anxiety** about several events/activities
T	**Tension** (muscle)
C	**Concentration** difficulty; mind going blank
H	**Hyperarousal**: **irritability**
E	**Energy loss**: easily fatigued
R	**Restlessness**: on edge
S	**Sleep** disturbance

⭐ **The 6 countable symptoms: restlessness, fatigue, poor concentration, irritability, muscle tension, sleep disturbance**.

Epidemiology	Boundaries	Course
Lifetime **~6%** adults; **3%** children; **11%** adolescents	Not due to substance/medical cause (**hyperthyroidism**)	Onset hard to specify; **presents in the 20s**
Mean onset **~33**; often presents in the 20s	Not better explained by panic, social anxiety, OCD, separation anxiety	Only **~⅓ seek psychiatric treatment**

Epidemiology	Boundaries	Course
⭐ **F:M ≈ 2:1**	⭐ **GAD vs MDD:** up to **80% of depressed patients report anxiety**; GAD symptoms must occur **IN THE ABSENCE of MDD**	**Chronic, possibly lifelong; fluctuates; worse with stress**
⭐ **Pure GAD is the exception**: the anxiety disorder **most often comorbid**	Anxiety only during depression = a symptom of depression	—

Treatment (slide 63): SSRI/SNRI + CBT; **buspirone** adjunct; benzodiazepine **short-term only**.



Slide 15: GAD criteria (WATCHERS)

2.4 Panic Attack and Panic Disorder (Slides 18–21) ⭐⭐

Panic attack:

- an **abrupt surge** of intense fear **peaking within minutes**, from a calm or anxious state
- ⭐ **≥ 4 of 13 symptoms**
- **a building block, not a diagnosis**

Cardiorespiratory	Autonomic & neuro	Cognitive
Palpitations	Sweating	**Fear of dying**
Shortness of breath	Dizziness	Fear of **"going crazy"/losing control**
Chest pain	Trembling	**Derealisation/depersonalisation**
Choking	Paraesthesias	—
—	Chills/heat; nausea/abdominal distress	—

Mnemonic *[added]*: STUDENTS + Fear of dying + 4 C's (8 + 1 + 4 = 13):

- **STUDENTS:**
  - **S**weating
  - **T**rembling
  - **U**nsteadiness/dizziness
  - **D**erealisation
  - **E**levated heart rate

    - **N**ausea
    - **T**ingling
    - **S**hortness of breath
- **Fear of dying**
- **4 C's:**
    - **C**razy (fear of losing control)
    - **C**hest pain
    - **C**hills/heat
    - **C**hoking

Panic disorder criteria (slide 19):

- ⭐ **recurrent, UNEXPECTED** panic attacks
- **≥ 1 attack** followed by **≥ 1 month** of:
    - **anticipatory anxiety** (worry about more attacks), and/or
    - a **maladaptive behaviour change** (avoidance)
- not due to a substance or medical condition (cardiac, hyperthyroid); not better explained by social anxiety, phobia, OCD, PTSD, separation anxiety
- ⭐ The **"out-of-the-blue"** quality distinguishes it from the **cued** attacks of phobias.

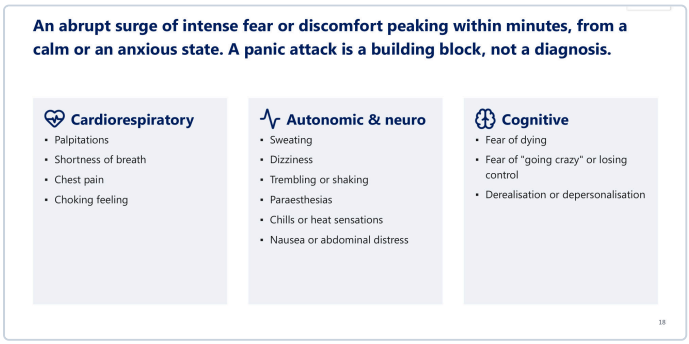
Panic cycle (slide 20):

1. **bodily sensation** (palpitation, dizziness)
2. **catastrophic thought** ("I'm having a heart attack")
3. **fear surges** (autonomic arousal)
4. **more sensations** confirm the fear → **PANIC**

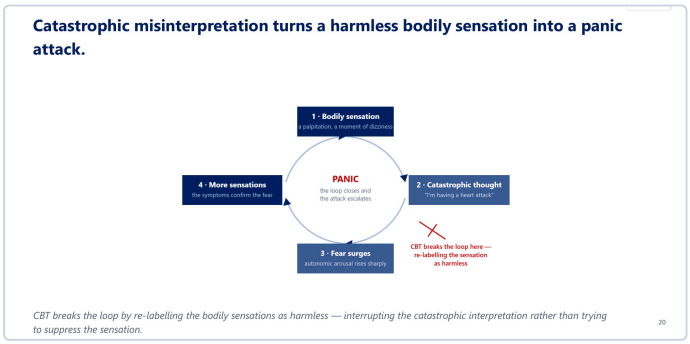
⭐ **CBT breaks the loop at step 2**, re-labelling the sensation as harmless.

Course	Prognosis	Management
Onset **late adolescence/early adulthood**; chronic	**30–40%** symptom-free long-term	⭐ **SSRI/SNRI + CBT**
⭐ **91% have another psychiatric disorder**	**~50%** mild residual symptoms	**Benzodiazepine short-term only (4–12 weeks)**, tapered slowly
⭐ **Two-thirds occur during or after MDD**	**10–20%** significant ongoing symptoms	⭐ **β-BLOCKERS ARE NOT USEFUL**

[added] Panic is provoked by **sodium lactate infusion, CO_2 inhalation, caffeine** (slide 62).



Slide 18: Panic attack, 4 of 13



Slide 20: The panic cycle

2.5 Agoraphobia (Slides 22–23) ⭐

The five situations:

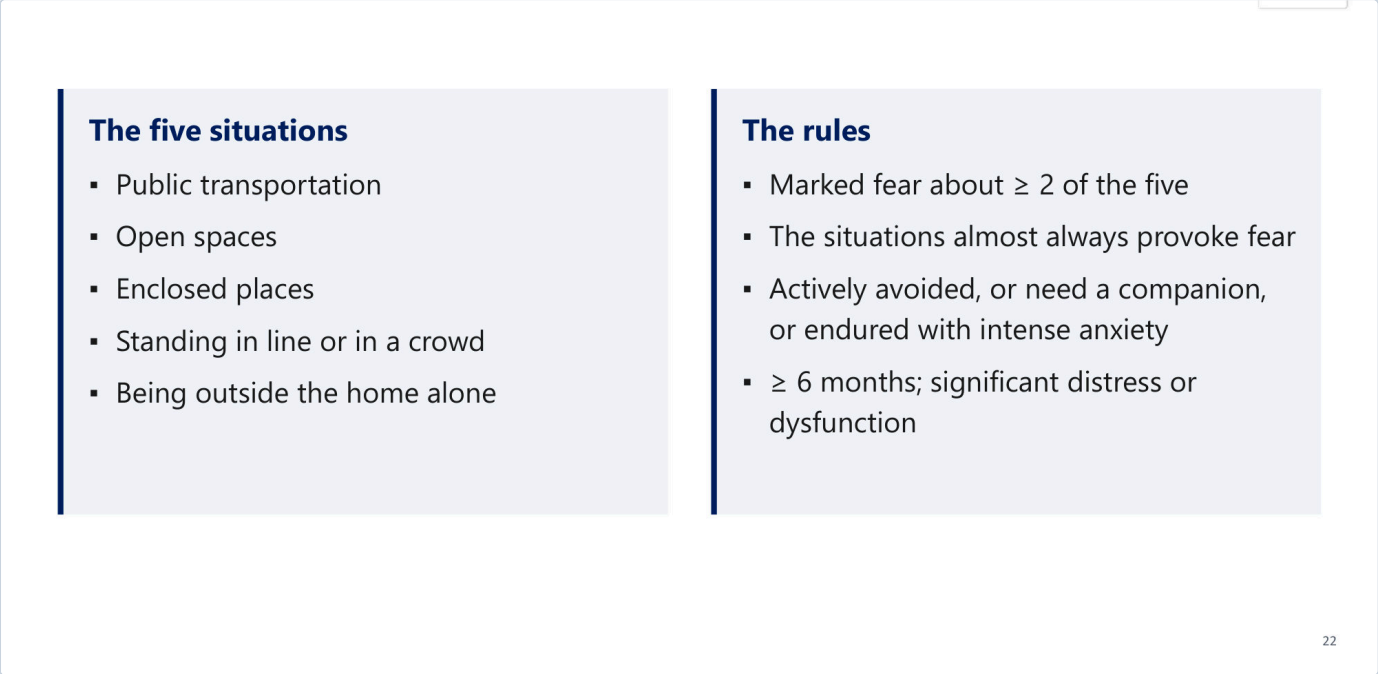
1. **public transport**
2. **open spaces**
3. **enclosed places**
4. **standing in line/crowd**
5. **outside the home alone**

Rules:

- ⭐ **marked fear of ≥ 2 of the 5**
- the situations almost always provoke fear
- **actively avoided, needs a companion, or endured with intense anxiety**
- **≥ 6 months**; distress/dysfunction

The feared consequence: that **escape might be difficult or help unavailable** if panic-like, incapacitating or embarrassing symptoms occur (e.g. **fear of falling in the elderly, incontinence**).

Course and management
Onset **~20** (later onset not uncommon)
⭐ **Chronic and unremitting without treatment**
⭐ **Agoraphobia WITHOUT panic is the most persistent form**
Treat as panic disorder: **SSRI/SNRI + exposure-based CBT**



Slide 22: Agoraphobia, 2 of 5 situations

2.6 Social Anxiety Disorder (Slides 24–25) ⭐

Criteria:

- marked fear of **≥ 1 social situation with possible scrutiny** (speaking, meeting people, **eating in public**)
- ⭐ fear of being **negatively evaluated or humiliated**
- avoided or endured with intense fear; out of proportion
- **≥ 6 months**
- **If a medical condition is present** (Parkinson's, obesity, disfigurement), the fear is **unrelated to it or clearly excessive**.

- **Specifier: ⭐ PERFORMANCE-ONLY type**.

Epidemiology	Prognosis	⭐ The pearl
Lifetime **3–13%**	**Half to two-thirds** respond in trials	**Generalised type → SSRI/SNRI + CBT**
Onset in the **teens** (range 5–35)	~Half of responders **fully remit**	⭐ **PERFORMANCE-ONLY → propranolol or benzodiazepine PRN before the event**
—	Continue **6–12 months**; CBT benefits persist **up to 5 years**	Contrast: **β-blockers NOT useful in panic disorder** (commonly examined)

2.7 Specific Phobia (Slides 26–28)

Criteria:

- marked fear of a **specific object/situation** that **almost always provokes immediate fear**
- avoided or endured with intense anxiety; **out of proportion**
- **≥ 6 months**

Epidemiology:

- lifetime **10–13%**; **F > M**
- onset **5–12 years**
- **animal and blood-injection-injury** types start earlier than situational/natural-environment types

⭐ **Blood-injection-injury phobia is unique:** it can cause a **VASOVAGAL FAINT** (**bradycardia and hypotension**) rather than tachycardia. It is managed with ⭐ **applied tension**.

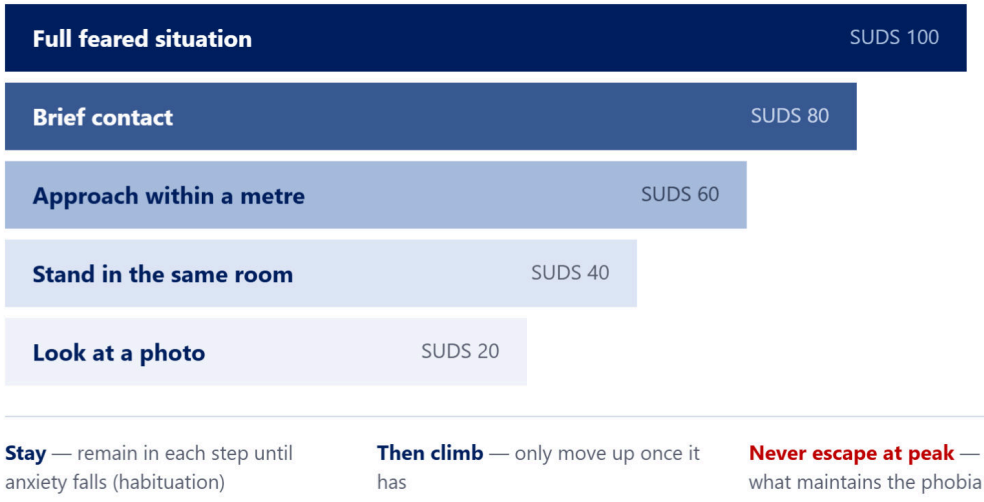
Exposure hierarchy / fear ladder (slide 27):

1. look at a photo (SUDS 20)
2. stand in the same room (40)
3. approach within a metre (60)
4. brief contact (80)
5. full feared situation (100)

Rules:

- **stay** at each step until anxiety falls (**habituation**)
- **then climb**
- ⭐ **never escape at the peak**: that is what maintains the phobia

Exposure is **graded and agreed**, not confrontational. Specific phobia treatment = **exposure therapy**; medication has a limited role.



The mechanism is habituation — which is why exposure is graded and agreed with the patient rather than confrontational.

27

Slide 27: The exposure hierarchy

Specific phobia vs agoraphobia (slide 28) ⭐

Presentation	Diagnosis
Fear of **airplanes and trains only**	**Specific phobia**
Fear of flying **because the plane will crash**	**Specific phobia**
Fear of **airplanes, elevators AND crowds**	**Agoraphobia**

- **Agoraphobia:** ≥ 2 situations, avoided because of **panic-like/embarrassing symptoms**.
- **Specific phobia: one** situation; fear **not related to panic-like symptoms**.
- ⭐ **Ask WHY**: the feared consequence assigns the diagnosis.

The distinction

- Agoraphobia: ≥ 2 situations, avoided because of panic-like or embarrassing symptoms
- Specific phobia: ONE situation, and the fear is not related to panic-like symptoms

Worked cases

- Fear of airplanes and trains only → specific phobia
- Fear of flying because the plane will crash → specific phobia
- Fear of airplanes, elevators AND crowds → agoraphobia

Ask WHY the situation is feared — the feared consequence, not the situation itself, assigns the diagnosis.

28

Slide 28: Specific phobia vs agoraphobia

2.8 Separation Anxiety Disorder (Slides 29–30)

Criteria:

- excessive fear of separation from an attachment figure; **≥ 3 features**:
  - distress on separation
  - worry about losing them or harm befalling them
  - worry an untoward event (getting lost, kidnapped) will cause separation
  - refusal to go out
  - fear of being alone or sleeping away from home
- ⭐ **duration > 4 weeks in children, > 6 months in adults**
- not better explained by autism, psychosis, agoraphobia, GAD

Across the lifespan:

- grouped with childhood disorders from DSM-III to DSM-IV-TR; ⭐ **DSM-5 recognises it in ADULTS**
- lifetime **4.1% in childhood, 6.6% in adults**
- ⭐ **most adults had first onset IN ADULTHOOD** (only ~⅓ childhood-onset)
- attachment figure: **parents** (children) → **spouse/friend** (adults)

> 🔁 **Recap: Anxiety disorders.**
> - Exclude caffeine, stimulants, withdrawal, thyroid, phaeochromocytoma, hypoglycaemia, cardiac/pulmonary causes.
> - **GAD:** ≥ 6 months + ≥ 3 of 6 (WATCHERS), outside MDD; F 2:1; most comorbid.
> - **Panic disorder:** unexpected attacks (≥ 4/13) + ≥ 1 month of worry/avoidance; **no β-blockers**.
> - **Agoraphobia:** ≥ 2 of 5, fear of no escape, ≥ 6 months; now separate from panic.
> - **Social anxiety:** scrutiny; **performance-only → propranolol PRN**.
> - **Specific phobia:** one object; BII → vasovagal, **applied tension**; exposure ladder.
> - **Separation anxiety:** > 4 weeks children / > 6 months adults; mostly adult-onset in adults.
>
> ▶️ **Up next: OCD and related disorders.**

2.9 OC-Related Disorders Map (Slide 32)

Unifying theme: repetitive thoughts and behaviours.

Disorder	Core
OCD	Obsessions, compulsions, or both
Body dysmorphic disorder	Preoccupation with a **perceived** appearance flaw
Hoarding disorder	Difficulty discarding, with clutter
Trichotillomania	Recurrent **hair-pulling** → hair loss
Excoriation	Recurrent **skin-picking** → lesions
Secondary	Substance-induced or due to a medical condition

2.10 OCD (Slides 33–41) ⭐⭐

Case (slides 33–34): intrusive images of leaving the stove on; checks it **exactly 7 times**. The checking is → ⭐ **B: a compulsion**.

- The intrusive image = **obsession** (thought); the neutralising check = **compulsion** (behaviour or mental act).
- ⭐ **Obsessions raise anxiety; compulsions temporarily relieve it.**

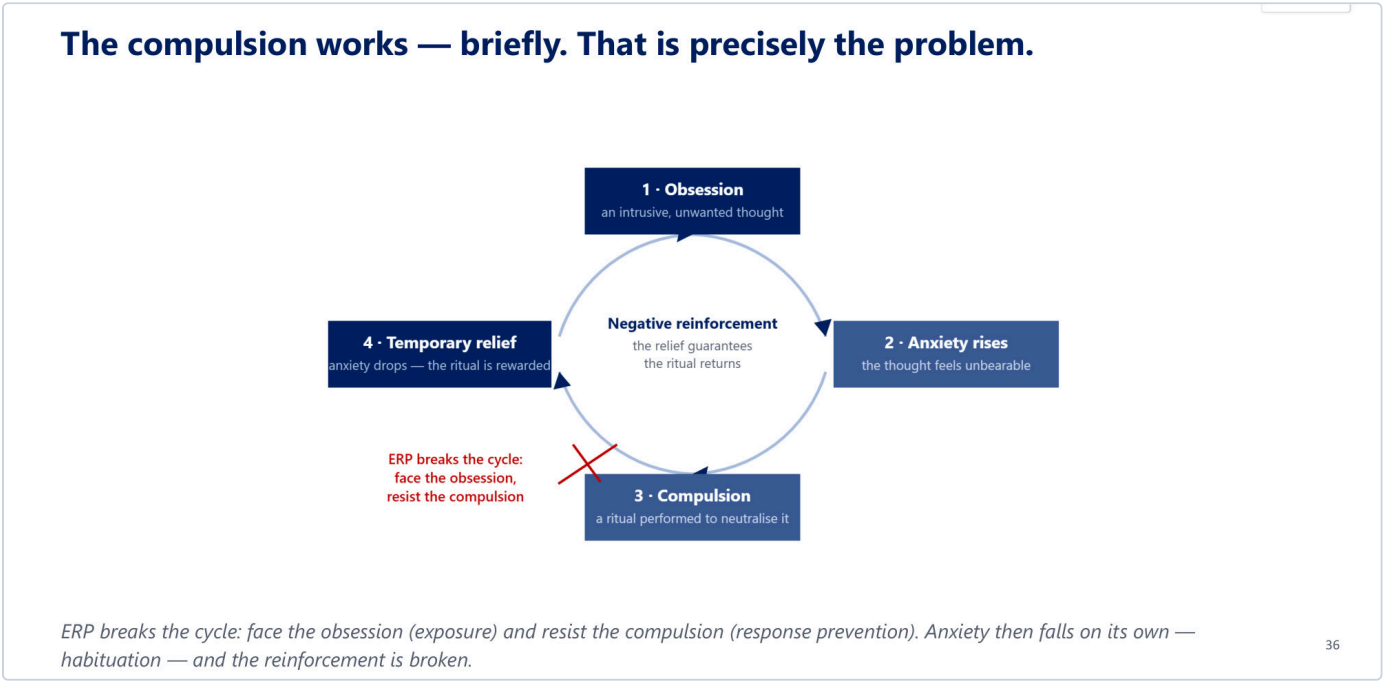
Obsessions	Compulsions
Recurrent, persistent **thoughts, urges or images**	Repetitive **behaviours or MENTAL ACTS** the person feels **driven** to perform
Intrusive and UNWANTED	In response to an obsession or **rigid rules**
Attempts to **ignore, suppress or neutralise** them	Aimed at **preventing/reducing distress**
—	**Not realistically connected**, or clearly **excessive**

The compulsion is a RESPONSE to the obsession: that link is the core of OCD.

The OCD vicious cycle (slide 36):

1. obsession
2. anxiety rises
3. compulsion
4. temporary relief → **negative reinforcement** → the ritual returns

⭐ **ERP** = **E**xposure (face the obsession) + **R**esponse **P**revention (resist the compulsion) → anxiety falls by **habituation**.



Slide 36: The OCD cycle

Common obsession → compulsion pairs (slide 37):

Obsession	Compulsion	Clinical clue
⭐ **Contamination**	**Washing, cleaning**	**Chapped hands**; avoids handshakes and **doctors**
Doubt (harm, safety)	**Checking** locks, stove, gas	Late leaving home
Symmetry / "just right"	**Ordering, arranging, counting**	Tasks never feel finished
⭐ **Forbidden/taboo thoughts**	**Mental rituals, reassurance-seeking**	**Hidden; high shame; HIGH SUICIDE RISK**

Obsession	Compulsion	Clinical clue
Fear of losing things	Saving	Now a separate **hoarding** diagnosis

Compulsions can be **purely MENTAL** (counting, praying, reviewing). Ask about both or you'll miss **"pure-O"**.

Obsession theme	Typical compulsion	Clinical clue
Contamination	Washing, cleaning	Chapped hands; avoids handshakes and doctors
Doubt (harm, safety)	Checking locks, stove, gas	Late leaving home; re-checks repeatedly
Symmetry / "just right"	Ordering, arranging, counting	Tasks never feel finished
Forbidden / taboo thoughts	Mental rituals, reassurance-seeking	Hidden; high shame; high suicide risk
Fear of losing things	Saving — now a separate hoarding diagnosis	Distinguish from hoarding disorder

Compulsions can be behavioural (washing, checking) or purely MENTAL (counting, praying, reviewing) — ask about both, or you will miss "pure-O" presentations.

37

Slide 37: Obsession → compulsion patterns

Threshold and insight (slide 38):

- ⭐ **Time-consuming: > 1 hour/day**, OR marked distress/dysfunction
- not due to a substance, medical or other psychiatric disorder

Insight specifier	Belief	Significance
Good/fair	Obsessional beliefs **probably NOT true**	The usual presentation; **best prognosis**
Poor	Beliefs **probably true**	Worse treatment response
⭐ **Absent/delusional**	**Completely convinced**	Worst prognosis; ⭐ **easily MISTAKEN FOR PSYCHOSIS**, but **treated as OCD**, not schizophrenia

Threshold: time-consuming — more than 1 hour a day — OR marked distress or dysfunction, and not attributable to a substance, medical or other psychiatric disorder.

Insight specifier	The patient believes...	Why it matters
Good or fair	The obsessional beliefs are probably NOT true	The usual presentation; best prognosis
Poor	The beliefs are probably true	Worse response to treatment
Absent / delusional	Completely convinced the beliefs are true	Worst prognosis — and easily MISTAKEN FOR PSYCHOSIS

Poor or absent insight predicts a worse prognosis — and delusional-level OCD is treated as OCD, not as schizophrenia.

38

Slide 38: OCD threshold and insight

Course (slide 39)	Good prognostic factors
⭐ **Mean onset ~19.5 years**; usually gradual	**Non-severe** symptoms
Untreated: **chronic, waxing and waning**	**No comorbid OCPD**
⭐ **Suicidal ideation ~50%; attempts ~25%**	Depressed/anxious mood rather than **schizotypal**
—	**Treatment compliance; family support**

Quality of life (slide 40):

- harm obsessions make relationships feel hazardous → withdrawal
- symmetry obsessions → projects never finish → school/job loss
- **contamination fears → avoid physicians; washing → dermatological damage**
- ⭐ symptoms **obstruct the cure**: medications may be seen as "**contaminated**" and refused

Treatment (slide 41) ⭐

First-line	If inadequate
⭐ **CBT with ERP**, medication, or both	Switch to another SSRI, ⭐ **clomipramine**, or venlafaxine
⭐ **SSRI at MAXIMUM dose for ≥ 6 weeks**	⭐ **Augment with an antipsychotic**: e.g. **risperidone 0.5–3 mg/day**
⭐ OCD often needs **HIGHER doses than depression**	SSRIs and clomipramine improve **40–60%**

⭐ **ERP is the single most effective intervention for OCD**; medication is an alternative or adjunct.

Neurobiology (slide 62):

- **cortico-striatal-thalamic loop** (caudate, basal ganglia); serotonin + glutamate
- ⭐ **PANDAS:** paediatric OCD and tics after **group-A strep** (autoimmune)
- OCD is associated with **Tourette's** and **Sydenham chorea** (*[added]* link to Rheumatic Fever)

2.11 Body Dysmorphic Disorder (Slides 42–44) ⭐

Criteria:

- preoccupation with **≥ 1 perceived defect**, **not observable or only slight** to others
- **repetitive behaviours** (**mirror-checking**, grooming, **skin-picking**, reassurance-seeking) or **mental acts** (comparing)
- marked distress/dysfunction
- ⭐ **not better explained by an eating disorder** (weight/fat concerns)
- ⭐ **Muscle dysmorphia** specifier: body "too small/not muscular enough"; often **missed in men**

Case (slide 43):

- **Ms. R, 28**, attractive, believes she is "ugly" and that others laugh at her
- began at **13** (nose "too fat", eyes "too far apart")
- withdrew, grades fell; picks blemishes, checks mirrors all day

Course	Management
Lifetime **~2%**, slightly more females	**Educate**; focus on **SUFFERING and impaired function**
Onset in **adolescence**; long undulating course; few symptom-free intervals	⭐ **Do NOT comment on their looks**: reassurance entrenches the preoccupation
⭐ **Suicide attempts 6–23× the general population**	**Mild–moderate: SSRI or CBT**; non-responders: **clomipramine**
—	⭐ **Maintain 3–4 years**

⭐ Patients seek **cosmetic/dermatological procedures**, which **rarely help and may worsen** the preoccupation.

2.12 Hoarding Disorder (Slides 45–46)

Criteria:

- persistent difficulty **discarding possessions, REGARDLESS OF VALUE**
- a perceived need to save; distress on discarding
- clutter congests living areas (uncluttered only via third parties)
- not due to a medical condition (**brain injury, CVA, Prader–Willi**)
- **Specify: excessive acquisition** (most cases) and **insight level**

Item	Detail
Prevalence	**~1.5%** (England); **M = F**
Trajectory	Emerges **age 11–15** → interferes by **mid-20s** → significant impairment by **mid-30s**
Course	Chronic, waxing and waning
⭐ **FIRST priority**	**SAFETY RISKS (fire, falls)**: **forcible action if children or elderly people are at risk**
Treatment	**CBT** (limited availability) or **SSRI** (esp. with comorbid mood/anxiety)

Hoarding is the one disorder here where **safety may force action before any therapeutic alliance exists**.

2.13 Trichotillomania and Excoriation (Slides 47–49)

Trichotillomania criteria:

- recurrent hair-pulling → hair loss

- **repeated attempts to stop**
- distress/dysfunction
- not dermatological or another mental disorder (e.g. BDD)

Case:

- **Ms. C, 27**: started at **11** at the nape → whole scalp, bald patches hidden by scarves
- high-functioning (master's)
- daily, often **automatic**; stopping causes **rising tension**

Two patterns	Complication	Treatment
Early–mid adolescence: ⭐ **F:M 10:1**, serious, chronic	⭐ **5–40% chew/swallow hair**	**No consensus**
Childhood onset: F = M, less serious, more common	⭐ **~⅓ of those develop BEZOARS** (hairballs), sometimes **surgical**	**SSRI, or CBT with habit reversal**; augment with an antipsychotic

⭐ **Ask every hair-pulling patient whether they swallow the hair.**

Excoriation (skin-picking):

- **criteria:** recurrent picking → lesions; attempts to stop; distress; **not due to a substance (e.g. cocaine)** or another disorder
- **clinical:** face, arms, hands; **onset at puberty**; **female predominance**; comorbid with OCD, BDD, trichotillomania
- **treatment: CBT with habit reversal ± SSRI**

⭐ **Trichotillomania and excoriation = BODY-FOCUSED REPETITIVE BEHAVIOURS.** The pull/pick is often **soothing**, not distress-driven like a classic compulsion.

> 🔁 **Recap: OC-related disorders.**
> - **OCD:** obsession (thought, raises anxiety) → compulsion (act/mental act, relieves it); > 1 h/day; insight specifier (absent ≠ psychosis); onset ~19.5; **SI 50%, attempts 25%**. **ERP** + high-dose SSRI ≥ 6 weeks → clomipramine → risperidone augmentation. PANDAS.
> - **BDD:** perceived flaw; don't comment on looks; SSRI/CBT, clomipramine; 3–4 years; suicide 6–23×.
> - **Hoarding:** regardless of value; safety first.
> - **BFRBs** (trichotillomania, excoriation): habit reversal; bezoars.
>
> ▶️ **Up next: Trauma- and stressor-related disorders.**

2.14 Acute Stress Disorder vs PTSD (Slides 52–55) ⭐⭐

Case (slides 52–53): 10 days after a car crash, a man has nightmares, flashbacks, hypervigilance and numbing, starting the day after → ⭐ **B: acute stress disorder**.

Time since trauma	Diagnosis
< 3 days	⭐ **No diagnosis**: acute distress is normal
3 days – 1 month	⭐ **Acute stress disorder** (remits within the month)
> 1 month	⭐ **PTSD** (same symptoms, persisting)

Both require **exposure to a traumatic event** and **significant impairment**. **Only the clock differs.**



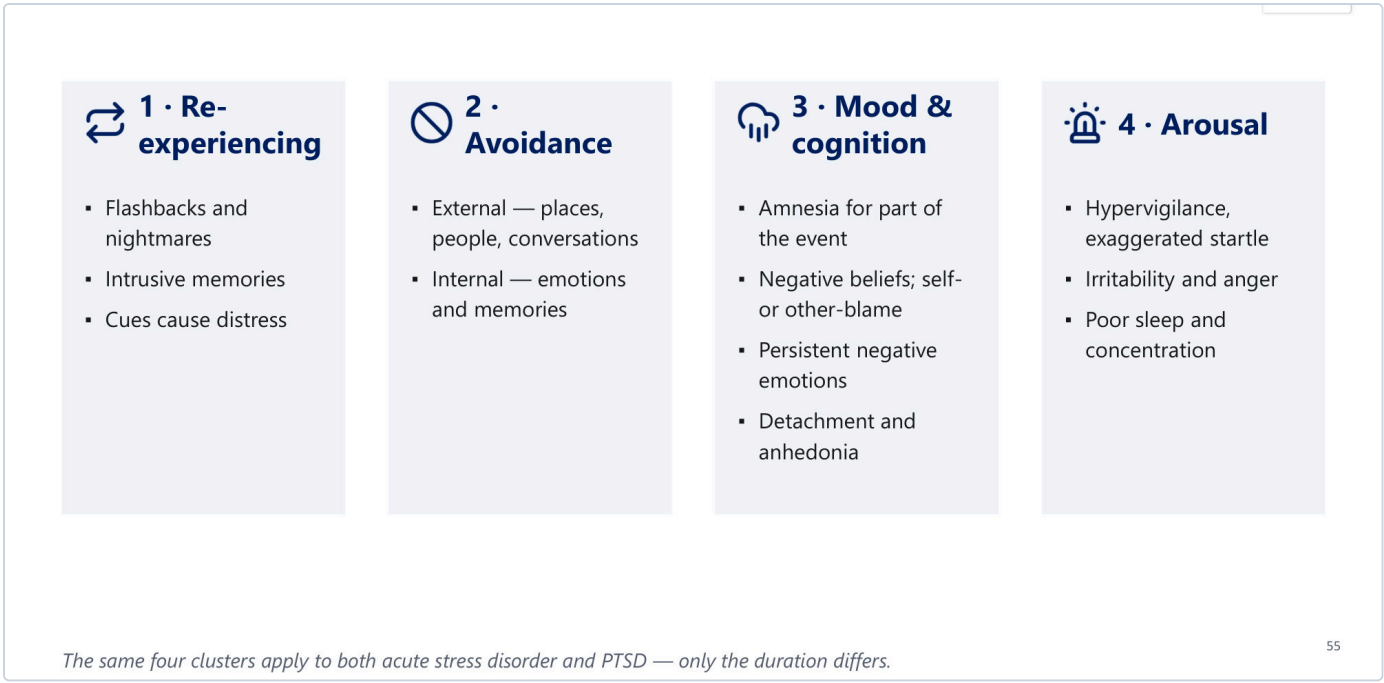
Slide 54: ASD vs PTSD timeline

The 4 symptom clusters (slide 55):

1. Re-experiencing	2. Avoidance	3. Mood & cognition	4. Arousal
Flashbacks, nightmares	**External**: places, people, conversations	**Amnesia** for part of the event	**Hypervigilance, exaggerated startle**
Intrusive memories	**Internal**: emotions, memories	Negative beliefs; **self/other-blame**	**Irritability, anger**
Cues cause distress	—	Persistent negative emotions; **detachment, anhedonia**	**Poor sleep and concentration**

Mnemonic *[added]*: "TRAUMA":

- **T**rauma exposure
- **R**e-experiencing
- **A**voidance
- **U**nable to function
- **M**onth (> 1 for PTSD)
- **A**rousal (+ negative mood/cognition)



Slide 55: The four PTSD clusters

Epidemiology & risk (slide 56)	Comorbidity & prognosis (slide 57)
⭐ **~8% lifetime** (general population); **up to 75%** in high-risk trauma-exposed groups; **5–15% subclinical**	**Two-thirds have ≥ 2 other disorders**: depression, substance use, anxiety, bipolar
⭐ **MOST IMPORTANT risk factor: severity, duration and proximity of the trauma** ("the **dose** of trauma")	Untreated: **~30% recover completely; 40% mild; 20% moderate; 10% unchanged/worse**
Also: single/divorced/widowed; socially withdrawn; low SES	**~50% recover by 1 year**
—	⭐ **Good prognosis: rapid onset, short duration (< 6 months), good premorbid function, strong support, no other disorders**

Neurobiology (slide 62):

- **amygdala over-active** in anxiety and PTSD
- ⭐ **PTSD: small hippocampus, LOW cortisol, HIGH CRF** (*[added]* contrast with depression's high cortisol)

Management (slide 58) ⭐

Psychotherapy: first-line	Pharmacotherapy
⭐ **TRAUMA-FOCUSED CBT** (exposure, cognitive processing therapy)	First-line: ⭐ **SSRIs (sertraline, paroxetine) or SNRI (venlafaxine)**
⭐ **EMDR**	⭐ **PRAZOSIN (α1-antagonist) for nightmares and hypervigilance**
Supportive, psychodynamic; couples/family therapy	Augment with an **SGA** in severe/resistant cases

⭐ **BENZODIAZEPINES ARE GENERALLY AVOIDED in PTSD**: they don't treat the core symptoms, carry dependence risk, and can worsen symptoms.

2.15 Adjustment Disorder (Slides 59–60)

Definition	Subtypes (predominant symptom)
Emotional/behavioural response to an **identifiable stressor** (financial, illness, relationship)	With **depressed mood**
⭐ **Onset WITHIN 3 MONTHS** of the stressor	With **anxiety**
⭐ **Remits within 6 months** of the stressor's removal	**Mixed** anxiety and depressed mood
Out of proportion or significant impairment; **not normal bereavement**	With **disturbance of conduct**
—	Mixed emotions and conduct · unspecified

Management:

- **supportive psychotherapy, CBT, group** (the mainstay)
- ⭐ **problem-solving on the STRESSOR itself**: often the **most useful and most often skipped**
- medication **only for target symptoms**, time-limited

⭐ **Adjustment disorder is a DIAGNOSIS OF EXCLUSION**: used when the reaction doesn't meet criteria for MDD, PTSD, etc.

> 🔁 **Recap: Trauma.**
> - < 3 days normal · 3 days–1 month **ASD** · > 1 month **PTSD**.
> - 4 clusters: re-experiencing, avoidance, mood/cognition, arousal.
> - PTSD ~8% (up to 75% if exposed); the **dose of trauma** matters most.
> - Small hippocampus, **low cortisol**, high CRF.
> - **TF-CBT/EMDR + SSRI/SNRI**; **prazosin** for nightmares; **avoid benzodiazepines**.
> - Adjustment: within 3 months, remits within 6 months; problem-solving; diagnosis of exclusion.

2.16 Neurobiology Pearls (Slide 62)

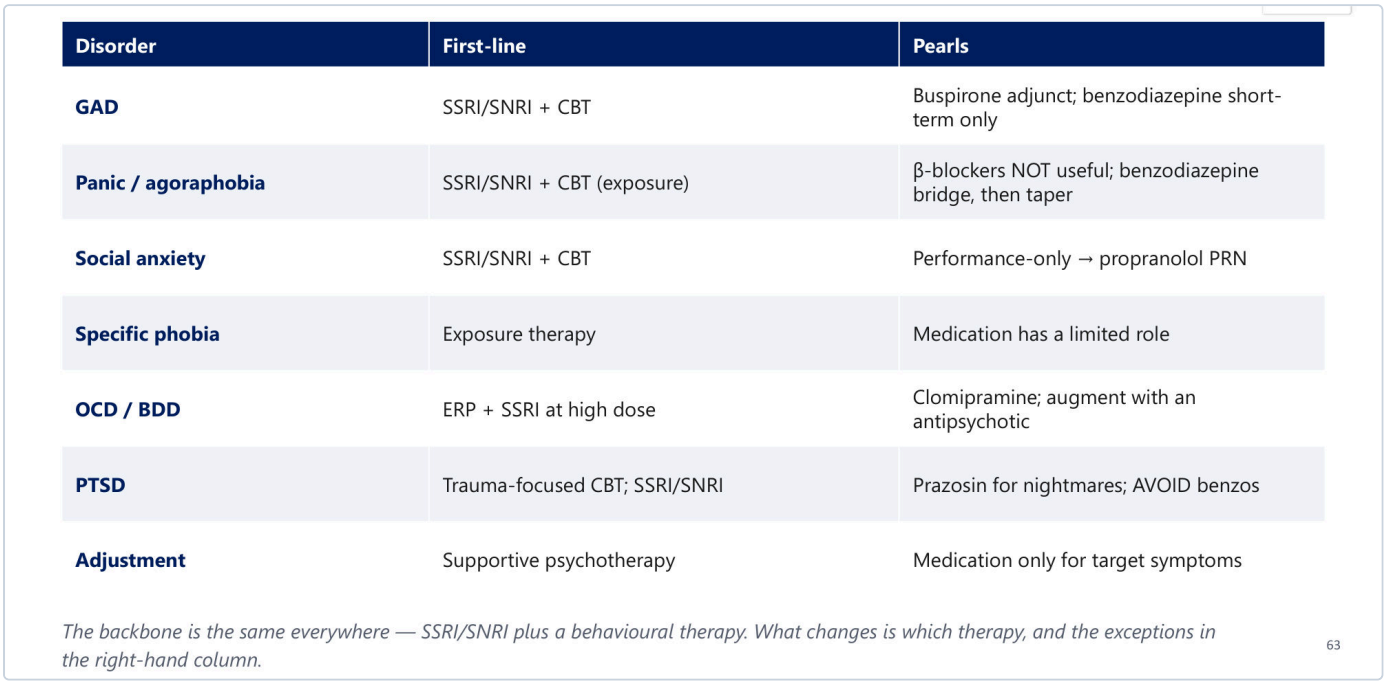
Circuits & chemistry	Tested triggers & links
⭐ **Amygdala**: the fear centre, over-active in anxiety/PTSD	⭐ **Panic provoked by sodium lactate, CO_2 inhalation, caffeine**
OCD: cortico-striatal-thalamic loop (**caudate**, basal ganglia); serotonin + glutamate	⭐ **PANDAS**: paediatric OCD + tics after **group-A strep**
⭐ **↓ GABA, ↓ serotonin, ↑ noradrenaline (locus coeruleus)**	OCD associated with **Tourette's** and **Sydenham chorea**
—	⭐ **PTSD:** small hippocampus, **low cortisol, high CRF**

2.17 Management at a Glance (Slide 63) ⭐⭐

Disorder	First-line	Pearls
GAD	SSRI/SNRI + CBT	**Buspirone** adjunct; benzodiazepine short-term only
Panic / agoraphobia	SSRI/SNRI + CBT (exposure)	⭐ **β-blockers NOT useful**; benzodiazepine bridge then taper

Disorder	First-line	Pearls
Social anxiety	SSRI/SNRI + CBT	⭐ **Performance-only → propranolol PRN**
Specific phobia	⭐ **Exposure therapy**	Medication has a limited role
OCD / BDD	⭐ **ERP + high-dose SSRI**	**Clomipramine**; augment with an antipsychotic
PTSD	⭐ **Trauma-focused CBT; SSRI/SNRI**	⭐ **Prazosin** for nightmares; **AVOID benzodiazepines**
Adjustment	**Supportive psychotherapy**	Medication only for target symptoms

The backbone everywhere is **SSRI/SNRI + a behavioural therapy**; what changes is **which therapy** and the **exceptions**.



Disorder	First-line	Pearls
GAD	SSRI/SNRI + CBT	Buspirone adjunct; benzodiazepine short-term only
Panic / agoraphobia	SSRI/SNRI + CBT (exposure)	β-blockers NOT useful; benzodiazepine bridge, then taper
Social anxiety	SSRI/SNRI + CBT	Performance-only → propranolol PRN
Specific phobia	Exposure therapy	Medication has a limited role
OCD / BDD	ERP + SSRI at high dose	Clomipramine; augment with an antipsychotic
PTSD	Trauma-focused CBT; SSRI/SNRI	Prazosin for nightmares; AVOID benzos
Adjustment	Supportive psychotherapy	Medication only for target symptoms

The backbone is the same everywhere — SSRI/SNRI plus a behavioural therapy. What changes is which therapy, and the exceptions in the right-hand column.

63

Slide 63: Management at a glance

2.18 Master Comparison and Duration Cheat-Sheet (Slides 64–65) ⭐⭐

Disorder	Key feature	Duration	Onset	Prevalence	Sex
GAD	Uncontrollable worry + ≥ 3 physical symptoms	**≥ 6 months**	~33	~6%	**F 2:1**
Panic disorder	Recurrent unexpected attacks	**≥ 1 month of worry**	Late teens–20s	~2–3%	F > M
Agoraphobia	≥ 2 situations; fear of no escape	**≥ 6 months**	~20	~1–2%	F > M
Social anxiety	Fear of scrutiny	**≥ 6 months**	Teens	3–13%	F > M
Specific phobia	One object/situation	**≥ 6 months**	5–12	10–13%	F > M
Separation anxiety	Fear of losing an attachment figure	**> 4 weeks (child) / > 6 months (adult)**	Child and adult	4–7%	F > M
OCD	Obsessions + compulsions	⭐ **> 1 h/day**	~19.5	~2–3%	⭐ **M = F**
PTSD	Trauma + 4 clusters	**> 1 month**	Any	~8%	F > M

⭐ **The two outliers: OCD** is the only one with **equal sex distribution**, and it is graded by **TIME SPENT**, not months.

Window	Disorders
3 days – 1 month	**Acute stress disorder** (< 3 days = normal distress)
≥ 1 month	**PTSD** · **panic disorder** (≥ 1 month of worry/avoidance) · **separation anxiety in children** (> 4 weeks)
≥ 6 months (the default)	**GAD · social anxiety · specific phobia · agoraphobia · separation anxiety in adults**
Off the axis	**OCD: > 1 h/day** · **Adjustment: onset within 3 months, remits within 6 months of the stressor ending**

Disorder	Key feature	Duration	Onset	Prevalence	Sex
GAD	Uncontrollable worry + ≥ 3 physical Sx	≥ 6 mo	~33	~6%	F 2:1
Panic disorder	Recurrent unexpected attacks	≥ 1 mo worry	late teens–20s	~2–3%	F > M
Agoraphobia	≥ 2 situations, fear no escape	≥ 6 mo	~20	~1–2%	F > M
Social anxiety	Fear of scrutiny	≥ 6 mo	teens	3–13%	F > M
Specific phobia	One object or situation	≥ 6 mo	5–12	10–13%	F > M
Separation anxiety	Fear of losing attachment figure	> 4 wk / > 6 mo	child & adult	4–7%	F > M
OCD	Obsessions + compulsions	> 1 hr/day	~19.5	~2–3%	M = F
PTSD	Trauma + 4 symptom clusters	> 1 mo	any	~8%	F > M

Note the two that differ from the pattern: OCD is the only one with equal sex distribution, and it is graded by TIME SPENT rather than months of duration.

64

Slide 64: Master comparison

3 days – 1 month ● **≥ 1 month** ● **≥ 6 months**

minimum duration required →

The trauma window

- ACUTE STRESS DISORDER — 3 days to 1 month after the trauma
- Under 3 days there is no diagnosis: acute distress is normal
- Same four symptom clusters as PTSD

The one-month group

- PTSD — beyond 1 month after the trauma
- PANIC DISORDER — ≥ 1 attack plus ≥ 1 month of worry or avoidance
- SEPARATION ANXIETY in children — > 4 weeks

The default

- GAD — more days than not
- SOCIAL ANXIETY · SPECIFIC PHOBIA · AGORAPHOBIA
- SEPARATION ANXIETY in adults
- Six months is the rule; the trauma disorders are the exception

Two sit off the axis. OCD is graded by TIME SPENT — more than an hour a day — not by months. ADJUSTMENT DISORDER is defined by its relation to the stressor: onset within 3 months, remitting within 6 months of the stressor going.

65

Slide 65: Duration cheat-sheet

2.19 Integrative Case: Mrs. A, 29 (Slides 66–67) ⭐

Presentation:

- teacher, **referred by cardiology** after 3 normal ECGs, troponins and an echo
- **1 year** of sudden pounding heart, chest tightness, breathlessness, dizziness, **tingling hands**; each **peaks in ~5 minutes**, passes in 30 minutes
- the **first came out of the blue** watching TV
- now **worries daily** about the next one
- **stopped driving on the ring road, using lifts, going to the mall alone**; goes only **with her sister**
- **5 cups of Arabic coffee/day**; poor sleep; weight loss; **TSH normal**; no low mood or trauma

Question	Model answer (lecture)
1. Diagnosis(es)?	⭐ **Panic disorder** (recurrent unexpected attacks + ≥ 1 month anticipatory anxiety/avoidance) **WITH agoraphobia as a separate diagnosis** (≥ 2 of 5 situations, needs a companion, ≥ 6 months)
2. Criteria?	Attacks peak within minutes with **≥ 4 of 13** symptoms; avoidance driven by fear that **escape would be difficult or help unavailable**
3. Organic causes?	**Hyperthyroidism** (TSH normal), **arrhythmia/MI** (cardiac work-up negative), ⭐ **caffeine** (5 cups/day is a genuine contributor)
4. Management?	**SSRI/SNRI + CBT with exposure**; benzodiazepine only as a **short bridge**, tapered; ⭐ **β-blockers NOT useful**; **cut caffeine**; treat sleep

Twist: if the attacks happened **only in lifts** → **specific phobia**, not panic disorder.

---

3. High-Yield Comparison Tables

Table A: Same Avoidance, Different Diagnosis (Ask WHY)

Avoids the mall because…	Diagnosis
Might have a panic attack and **can't escape/get help**	**Agoraphobia**
Might be **judged or humiliated**	**Social anxiety**
Afraid of **one specific thing** there (e.g. lifts, for their own sake)	**Specific phobia**
Fear of **contamination** from crowds	**OCD**
It **reminds them of a trauma** there	**PTSD**
Fear of being **separated** from a spouse	**Separation anxiety**

Table B: β-Blockers and Benzodiazepines Across the Chapter

Disorder	β-blocker	Benzodiazepine
Performance-only social anxiety	✅ **Propranolol PRN**	✅ PRN alternative
Panic disorder	❌ **Not useful**	Short bridge (4–12 weeks), taper
GAD	—	Short-term only
PTSD	—	❌ **Avoid**
[added] Akathisia (Psychosis lecture)	✅ Propranolol	—

Table C: Obsession vs Compulsion vs Delusion vs BFRB

	Obsession	Compulsion	Delusion	BFRB (trich/excoriation)
Nature	Thought/urge/image	Behaviour or mental act	Fixed false belief	Pulling/picking
Ego	**Dystonic**	Driven, often resisted	Syntonic (held as true)	Often **soothing**, automatic
Effect on anxiety	**Raises**	**Relieves** briefly	—	Relieves tension
Insight	Usually kept	—	**Absent**	Variable
Rx	ERP + SSRI	ERP	Antipsychotic	**Habit reversal** ± SSRI

Table D: Trauma Timeline

Time	Diagnosis
< 3 days	Normal acute distress
3 days – 1 month	Acute stress disorder
> 1 month	PTSD
Stressor (non-traumatic), onset ≤ 3 months, resolves ≤ 6 months after it ends	Adjustment disorder

Table E: Cortisol Across Disorders *[added synthesis]*

Disorder	Cortisol / HPA
Depression	↑ **Cortisol**, dexamethasone non-suppression
PTSD	↓ **Cortisol**, ↑ CRF, small hippocampus
Cushing's (Adrenal I)	↑ Cortisol (organic) → depression/psychosis

---

4. Rules, Exceptions, Patterns

- **Every diagnosis:** not due to substance/medical cause, plus significant distress/impairment.
- **The feared consequence assigns the diagnosis.**
- **≥ 6 months is the default**; exceptions: panic (1 month of worry), ASD (3 days–1 month), PTSD (> 1 month), separation anxiety in children (> 4 weeks), **OCD (> 1 h/day)**, adjustment (3/6-month rule).
- **GAD needs ≥ 3 of 6 in adults, 1 in children**, and must occur outside MDD.
- **Agoraphobia and panic are separate diagnoses** since DSM-5.
- **β-blockers:** yes for performance anxiety; **no** for panic.
- **Benzodiazepines:** short-term bridge only; **avoid in PTSD**.
- **Blood-injection-injury phobia → vasovagal** (bradycardia) → **applied tension**.
- **OCD = M = F**; all the others F > M (hoarding M = F too).
- **OCD needs higher SSRI doses** and ≥ 6 weeks; **clomipramine**; risperidone augmentation.
- **Absent-insight OCD is still OCD**, not psychosis.
- **OCD/BDD suicide risk is high**: OCD SI 50%/attempts 25%; BDD attempts 6–23×.
- **Don't reassure about looks in BDD**; avoid cosmetic surgery.
- **Hoarding: safety before alliance.**
- **Trichotillomania: ask about swallowing hair (bezoar).**

- **PTSD: low cortisol; prazosin for nightmares.**
- **Adjustment = diagnosis of exclusion**; treat the stressor.
- **Aim for function, not zero anxiety** (Yerkes–Dodson).

---

5. Clinical Correlations

- **Cardiology:** chest pain, palpitations, normal troponin/echo → panic disorder. But exclude **arrhythmia, MI, PE** first.
- **Endocrine:**
  - **hyperthyroidism, phaeochromocytoma, hypoglycaemia** mimic anxiety
  - **thyroxine over-replacement** causes anxiety
- **Respiratory: salbutamol/theophylline** cause anxiety; asthma/COPD dyspnoea overlaps with panic.
- **Dermatology:** contamination OCD → **chapped hands**; BDD → seeks dermatology/cosmetic care; excoriation lesions; **trichotillomania** (vs alopecia areata).
- **Surgery/GI: trichobezoar** in hair swallowers.
- **Paediatrics/Rheumatology: PANDAS** (OCD/tics after group-A strep); **Sydenham chorea** (Rheumatic Fever lecture) is associated with OCD.
- **Phlebotomy/vaccination:** BII phobia → vasovagal syncope; use **applied tension**, lying down.
- **Addiction:** alcohol/benzodiazepine **withdrawal** presents as anxiety.
- **Saudi context: Arabic coffee** = caffeine; taboo/religious obsessions (scrupulosity) hidden by shame.

---

6. Rapid Revision (Last-Day)

1. **Fear** = imminent threat; **anxiety** = future threat. Yerkes–Dodson: aim for function.
2. **Three DSM-5 chapters:** anxiety · OC-related · trauma/stressor.
3. **Exclude:**
   - caffeine, stimulants, sympathomimetics, bronchodilators, thyroxine, steroids
   - **withdrawal** (alcohol, benzodiazepines, opioids, nicotine)
   - hyperthyroidism, **phaeochromocytoma**, hypoglycaemia, arrhythmia, MI, PE, asthma/COPD, seizure, vestibular disease
4. **GAD:**
   - ≥ 6 months, more days than not, hard to control
   - **≥ 3 of 6** (restlessness, fatigue, concentration, irritability, tension, sleep); **1 in children**
   - ~6%, F 2:1, mean onset 33, most comorbid
   - outside MDD
   - SSRI/SNRI + CBT; buspirone
5. **Panic attack:** peaks within minutes; ≥ 4 of 13.
6. **Panic disorder:**
   - recurrent **unexpected** attacks + **≥ 1 month** worry/avoidance
   - **91% comorbid**; ⅔ with/after MDD
   - 30–40% symptom-free; 50% mild; 10–20% significant
   - SSRI/SNRI + CBT; benzodiazepine 4–12 weeks; **no β-blockers**
7. **Agoraphobia:**
   - **≥ 2 of 5** (transport, open, enclosed, crowds/lines, alone outside)
   - fear of no escape/help; ≥ 6 months

    - without panic = most persistent
8. **Social anxiety:** scrutiny/humiliation; 3–13%; teens; **performance-only → propranolol PRN**.
9. **Specific phobia:** one object; 10–13%; onset 5–12; **BII → vasovagal → applied tension**; **exposure ladder** (stay, climb, never escape at the peak).
10. **Separation anxiety:** ≥ 3 features; **> 4 weeks children, > 6 months adults**; adults mostly adult-onset.
11. **OCD:** - obsession raises anxiety; compulsion relieves it - **> 1 h/day**; insight good/poor/absent (absent ≠ psychosis) - onset 19.5; **M = F**; **SI 50%, attempts 25%** - **ERP**; SSRI max dose ≥ 6 weeks; clomipramine; risperidone augmentation - PANDAS; caudate
12. **BDD:** perceived flaw; mirror-checking; muscle dysmorphia; ~2%; **suicide attempts 6–23×**; don't comment on looks; SSRI/CBT → clomipramine; **3–4 years**.
13. **Hoarding:** regardless of value; ~1.5%, M = F; 11–15 → mid-30s; **safety first**; CBT/SSRI.
14. **Trichotillomania:** adolescent F:M 10:1; 5–40% swallow → ⅓ **bezoars**; habit reversal/SSRI.
15. **Excoriation:** puberty, female; habit reversal ± SSRI. **BFRBs are soothing.**
16. **ASD 3 days–1 month; PTSD > 1 month**; < 3 days = normal.
17. **PTSD 4 clusters:** re-experiencing, avoidance, negative mood/cognition, arousal.
18. **PTSD:** - ~8% (up to 75% exposed); **dose of trauma** matters most - ⅔ have ≥ 2 comorbidities; 50% recover by 1 year - 30/40/20/10 outcome split
19. **PTSD Rx: TF-CBT, EMDR**; sertraline/paroxetine/venlafaxine; **prazosin** for nightmares; SGA augmentation; **avoid benzodiazepines**.
20. **PTSD biology:** amygdala ↑, small hippocampus, **low cortisol, high CRF**.
21. **Adjustment:** onset ≤ 3 months, remits ≤ 6 months; subtypes; problem-solving; diagnosis of exclusion.
22. **Panic provocation:** lactate, CO_2, caffeine. **↓ GABA, ↓ 5-HT, ↑ NA (locus coeruleus).**

---

7. Exam-Style Questions

Q1 (lecture, slides 12–13). A 34-year-old woman has uncontrollable worry about work, children and health, with restlessness, poor sleep and muscle tension. Minimum duration for GAD? A. 2 weeks · B. 1 month · C. 3 months · **D. 6 months, more days than not**

> **Answer: D** (lecture key). GAD needs ≥ 6 months of excessive, hard-to-control worry + ≥ 3 of 6 physical symptoms.

Q2 (lecture, slides 33–34). A patient tormented by intrusive images of having left the stove on checks it exactly 7 times before leaving. The checking is a… A. Obsession · **B. Compulsion** · C. Delusion · D. Tic

> **Answer: B** (lecture key).
> - The image is the obsession; the neutralising check is the compulsion.
> - Obsessions raise anxiety; compulsions temporarily relieve it.

Q3 (lecture, slides 52–53). Ten days after a car crash, a man has nightmares, flashbacks, hypervigilance and emotional numbing that began the day after. Diagnosis now? A. PTSD · **B. Acute stress disorder** · C. Adjustment disorder · D. GAD

Answer: B (lecture key).

- 3 days–1 month = ASD.
- Beyond 1 month = PTSD. Same clusters, different clock.

Q4. A 22-year-old student is terrified of giving her thesis presentation next week; she is otherwise comfortable socially. Best pharmacological option? A. Daily SSRI for 12 months · **B. Propranolol PRN before the presentation** · C. Clomipramine · D. Prazosin

Answer: B (slides 25, 63, 68).

- Performance-only social anxiety → **propranolol PRN** (or a benzodiazepine PRN).
- β-blockers are **not** useful in panic disorder.

Q5. A 27-year-old with long-standing hair-pulling presents with epigastric pain, early satiety and a palpable mass. What must you ask? A. Family history of alopecia · **B. Whether she chews or swallows the hair** · C. Thyroid symptoms · D. Recent stressors

Answer: B (slide 48).

- 5–40% swallow hair; ~⅓ of those develop **trichobezoars**, sometimes needing surgery.

Lecture board one-liners (slide 68):

#	Question	Answer
1	Panic-like symptoms 3 weeks post-trauma: ASD or PTSD?	**ASD** (< 1 month); PTSD > 1 month
2	Most effective therapy for OCD?	**ERP**; SSRIs often at higher doses
3	Drug for performance-only social anxiety?	**Propranolol PRN**; not useful in panic
4	Agoraphobia: how many situations, and why avoided?	**≥ 2 of 5**; escape/help hard if panic-like symptoms occur
5	PTSD drug for nightmares?	**Prazosin** (α1-antagonist)
6	Paediatric OCD and tics after strep?	**PANDAS**

---

8. Exam Pitfalls & Traps

Trap	Correct thinking
Labelling the situation (mall, plane) as the diagnosis	❌ Ask **WHY**: the feared consequence decides
Fear of planes + trains only = agoraphobia	❌ Specific phobia (agoraphobia needs ≥ 2 situations **feared because of no escape**)
Panic disorder includes agoraphobia	❌ **Separate diagnoses** since DSM-5
β-blockers for panic disorder	❌ **Not useful**; only for performance-only social anxiety
Benzodiazepines long-term for anxiety	❌ Short-term bridge, tapered; **avoid in PTSD**
GAD diagnosed during an MDE	❌ GAD symptoms must occur outside MDD
GAD needs 3 symptoms in children	❌ **Only 1** in children
PTSD at 10 days	❌ **ASD** (3 days–1 month)
PTSD at day 2	❌ < 3 days = **normal acute distress**

Trap	Correct thinking
OCD duration ≥ 6 months	❌ OCD is graded by **time > 1 h/day** (or distress)
OCD with absent insight = schizophrenia	❌ Still **OCD**: treat as OCD
Checking = obsession	❌ Checking is the **compulsion** (thought = obsession)
Compulsions are always visible	❌ **Mental** compulsions ("pure-O") exist
OCD at a standard antidepressant dose for 2 weeks = failure	❌ **Max dose ≥ 6 weeks** before calling it a failure
Low suicide risk in OCD	❌ **SI ~50%, attempts ~25%**
Reassure the BDD patient they look fine	❌ Reassurance entrenches it; focus on suffering/function
BDD → refer for cosmetic surgery	❌ Rarely helps; may worsen
Hoarding = collecting valuables	❌ Difficulty discarding **regardless of value**
Engage before acting in hoarding	⚠️ **Safety first** (fire, falls); force if children/elderly at risk
BII phobia → tachycardia	❌ **Vasovagal** (bradycardia, hypotension) → applied tension
PTSD has high cortisol like depression	❌ PTSD: **low cortisol, high CRF**
Adjustment disorder when full MDD criteria are met	❌ Diagnose **MDD**; adjustment is a diagnosis of exclusion
Goal = no anxiety	❌ Goal = **function** (Yerkes–Dodson)